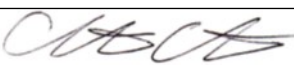


**Guideline Title: Pediatric Obstructed Airway -
Foreign Body Aspiration**

Guideline Number: GUID-3203-PED011

Issue date: 09/09/2014	Replaces Dept. Policy:
Revision dates: 4/30/24	Developed by: Air Care Team
Approved by: Dr. Christopher Hunter, MD Air Care Team Medical Director	Approved by:
Signature: 	Signature:
Department Numbers: 3203	

- I. **PURPOSE:** To remove the obstruction and maintain patent airway. To promote appropriate oxygenation and ventilation
- II. **PATHOPHYSIOLOGY:**

Aspiration of a foreign body may cause airway obstruction. Foreign bodies lodged in the esophagus will present differently from those lodged in the trachea or main stem bronchi. Other history may include the sudden onset of coughing or wheezing, associated with an episode of choking.

 - If object lodged at level of the glottis: hoarseness, cough, aphonia, and decreased breath sounds.
 - If object lodged below cords and in the trachea: persistent cough, decreased breath sounds bilaterally, palpable vibration over the trachea (object acts as a ball valve with respirations).
 - If object lodged in bronchus: wheezing & coughing. The object may be lodged in the right main stem so potentially have asymmetrical chest movement & decreased breath sounds on right side.
 - If chest X-ray is obtained, assess distal hyperinflation secondary to air trapping. The films will show a hyperexpanded lung on the involved side on the inspiratory film and a shift of the mediastinal structures to the uninvolved side on the expiratory film.
 - If object lodged in esophagus: stridor may be present
- III. **DEFINITIONS:**
- IV. **DEPARTMENT GUIDELINE:**
 - A. Assess and manage airway, breathing, and circulation per *GUID3203-PED001 – Pediatric General Management*.
 - B. Identify partial versus complete airway obstruction. If a partial airway obstruction exists, assess the patient's ability to maintain the airway and adequacy of ventilation.
 - C. If conscious: Follow AHA guidelines for obstructed airway
 - i. If infant (< 1 year)
 1. Give 5 back slaps and 5 chest thrusts
 2. Repeat until effective or patient becomes unresponsive
 - ii. If 1-8 years of age
 1. Give abdominal thrusts/Heimlich maneuver
 2. Repeat until effective or patient becomes unresponsive
 - D. If Unresponsive

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- i. If infant (< 1 year)
 1. Perform a tongue-jaw lift and if you see object remove it.
 2. Using laryngoscope and Magill forceps, attempt direct visualization for removal of object
 3. Open airway and try to ventilate if still obstructed. Reposition the head and try to ventilate again
 4. Give chest compressions at a rate of at least 100 per minute
 5. Repeat the previous 4 steps
- ii. If above steps are ineffective, initiate the following
 1. Attempt to establish an airway with tracheal intubation per *GUID3203-G008 - Rapid Sequence Intubation (RSI) for Endotracheal (ET) Intubation*.
 2. If attempts at retrieval and/or positive pressure ventilation are unsuccessful, a cricothyrotomy should be performed per *GUID3203-PR007 - Cricothyrotomy – Surgical*. An obstruction distal to the cricothyroid membrane may render a cricothyrotomy ineffective.
 3. If the object is lodged below the cords and in the trachea, it may be able to be displaced distally by pushing it into the right main stem which allows for ventilation of one lung. The tracheal tube should then be pulled back into the trachea to ventilate the left lung.
- iii. Follow Advanced Cardiac Guidelines.

V. DOCUMENTATION:

- A. EMS Charts

VI. REFERENCES:

- A. Pediatric Advanced Life Support Manual, 2020. American Heart Association.